

# Urination Pain - Male - Male (Child)

Algorithm 1484 | Child (0-17 years) | Gender at birth: Male

## Definition

- UAT-only adult and pediatric pathway for dysuria in patients with male urinary/reproductive anatomy, distinguishing sepsis, pyelonephritis, retention/obstruction, acute scrotal pain, prostatitis, trauma and urethral/STI causes from possible lower UTI.
- Child patient.
- Male.

## Initial assessment

1. [OPEN\_TEXT] Describe burning or pain, frequency, urgency, urine amount/flow, hesitancy, blood/cloudiness/odor, lower abdominal or perineal pain, and when symptoms began.
2. [TEMPERATURE] What is the temperature if measured, and are there rigors, flank/back pain, nausea, vomiting, confusion, weakness or reduced urine?
3. [YES\_NO] Is there sudden or severe testicular/scrotal pain or swelling, a high-riding or unusually positioned testis, genital injury, or groin/abdominal pain?
4. [YES\_NO] Is there inability to urinate, painful bladder fullness, severe one-sided pain, recent urinary/pelvic trauma or procedure, or catheter blockage/dislodgement?
5. [YES\_NO] Is there urethral discharge, penile pain/sores, pain with ejaculation, perineal/rectal pain, or possible STI exposure?
6. [YES\_NO] For a child or adolescent, is there fever, vomiting, poor feeding, reduced wet nappies/urine, new wetting, genital symptoms/injury, marked distress, or any privacy or safeguarding concern?
7. [CAREGIVER\_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

## Triage and redirect questions

### **1. Redirect → Flank Pain**

Is pain in the side or back the main concern rather than pain primarily during urination?

Pain focused in the side or back belongs in the Flank Pain pathway.

### **1. Disposition level 100**

Is there confusion, difficult waking, fainting, fast or difficult breathing, mottled/pale skin, severe weakness, very high or low temperature, inability to keep fluids down, very low urine output, severe or rapidly worsening pain, or a seriously unwell infant or child; or sudden severe testicular/scrotal pain or swelling?

These may indicate sepsis, severe pyelonephritis, acute kidney injury or another time-critical illness. Sudden severe unilateral scrotal pain/swelling may be torsion and requires immediate surgical assessment; urinary symptoms must not delay that pathway.

### **1. Disposition level 70**

With no emergency feature, is there any dysuria in a male, or fever/rigors, flank/back/perineal pain, vomiting, blood, reduced flow or retention, catheter, diabetes/immunosuppression, urinary abnormality, recurrent symptoms, genital discharge/sores, or no improvement within 48 hours of clinician-prescribed treatment?

Dysuria in male anatomy requires in-person assessment and urine testing to distinguish UTI, pyelonephritis, prostatitis, urethritis/STI, obstruction and other causes. Children need an age-specific pathway; adolescents need confidential, developmentally appropriate history and safeguarding assessment.

## Advice

### **Qatar urinary or acute-scrotal emergency response**

- Call Qatar emergency services on 999 and do not allow self-driving. Keep the person safely supervised. Do not give food or drink when urgent surgery may be needed for sudden severe scrotal pain, or force fluids if drowsy, vomiting, unable to swallow or unable to urinate. Do not manipulate the testis or catheter and do not use leftover antibiotics. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening confusion; breathing difficulty; collapse; very low urine; increasing scrotal pain/swelling.

### **Same-day urinary and genital assessment**

- Arrange same-day in-person assessment through an approved Qatar adult or pediatric pathway for urine sampling before antibiotics when feasible and clinician-directed genital, prostate or STI testing as indicated. Adolescents should be offered developmentally appropriate private time, within Qatar consent, confidentiality and safeguarding policy. Do not start leftover antibiotics or use fixed-dose/timed analgesia without an approved Qatar medicine pathway. Call 999 for sudden severe scrotal pain, retention, sepsis features or deterioration. Exact non-emergency destinations are local approval required.
- Call Back If: fever or rigors; testicular pain develops or worsens; vomiting; cannot urinate; blood in urine; becomes very unwell.

## First aid

- Call Qatar emergency services on 999 and do not allow self-driving. Keep the person safely supervised. Do not give food or drink when urgent surgery may be needed for sudden severe scrotal pain, or force fluids if drowsy, vomiting, unable to swallow or unable to urinate. Do not manipulate the testis or catheter and do not use leftover antibiotics. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

## References

- Qatar Ministry of Public Health, "Healthcare Services in Qatar", <https://sportandhealth.moph.gov.qa/EN/faninfo/Pages/HealthcareServicesInQatar.aspx> (accessed 2026-07-25)
- NICE NG109, "Urinary tract infection (lower): antimicrobial prescribing" (updated 2026)
- NICE NG111, "Pyelonephritis (acute): antimicrobial prescribing" (accessed 2026-07-25)

- CDC, "Urethritis and Cervicitis - STI Treatment Guidelines" and "Adolescents - STI Treatment Guidelines" (accessed 2026-07-25)

## Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

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